

Screening for Depression in Adults

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Tracking Information

Publication Number

100-3

Manual Section Number

210.9

Manual Section Title

Screening for Depression in Adults

Version Number

1

Effective Date of this Version

10/14/2011

Implementation Date

12/27/2011

Description Information

Benefit Category

Additional Preventive Services

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

A. General

Among persons older than 65 years, one in six suffers from depression. Depression in older adults is estimated to occur in 25% of those with other illness including cancer, arthritis, stroke, chronic lung disease, and cardiovascular disease. Other stressful events, such as the loss of friends and loved ones, are also risk factors for depression. Opportunities are missed to improve health outcomes when mental illness is under-recognized and under-treated in primary care settings.

Older adults have the highest risk of suicide of all age groups. These patients are important in the primary care setting because 50-75% of older adults who commit suicide saw their medical doctor during the prior month for general medical care, and 39% were seen during

the week prior to their death. Symptoms of major depression that are felt nearly every day include, but are limited to, feeling sad or empty; less interest in daily activities; weight loss or gain when not dieting; less ability to think or concentrate; tearfulness, feelings of worthlessness, and thoughts of death or suicide.

Based upon authority to cover “additional preventive services” for Medicare beneficiaries if certain statutory requirements are met, the Centers for Medicare & Medicaid Services (CMS) initiated a new national coverage analysis on screening for depression in adults. Screening for depression in adults is recommended with a grade of B by the U.S. Preventive Services Task Force (USPSTF) and is appropriate for individuals entitled to benefits under Part A and Part B.

Therefore, effective October 14, 2011, CMS will cover annual screening for depression for Medicare beneficiaries in primary care settings that have staff-assisted depression care supports in place to assure accurate diagnosis, effective treatment, and follow-up. Various screening tools are available for screening for depression. CMS does not identify specific depression screening tools. Rather, the decision to use a specific tool is at the discretion of the clinician in the primary care setting.

Coverage is limited to screening services and does not include treatment options for depression or any diseases, complications, or chronic conditions resulting from depression, nor does it address therapeutic interventions such as pharmacotherapy, combination therapy (counseling and medications), or other interventions for depression.

Indications and Limitations of Coverage

B. Nationally Covered Indications

Effective for claims with dates of service on or after October 14, 2011, CMS will cover annual screening up to 15 minutes for Medicare beneficiaries when staff-assisted depression care supports are in place to assure accurate diagnosis, effective treatment, and follow-up. At a minimum level, staff-assisted supports consist of clinical staff (e.g., nurse, physician assistant) in the primary care setting who can advise the physician of screening results and who can facilitate and coordinate referrals to mental health treatment.

C. Nationally Non-Covered Indications

Screening for depression is non-covered when performed more than one time in a 12-month period. In addition, self-help materials, telephone calls, and web-based counseling are not separately reimbursable by Medicare and are not part of this NCD.

D. Other

Medicare coinsurance and Part B deductible are waived for this preventive service.

(This NCD last reviewed October 2011.)

Claims Processing Instructions

[TN 2359 \(Medicare Claims Processing\)](#) 

Transmittal Information

Transmittal Number

139

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/downloads/R139NCD.pdf> [↗](#)

Revision History

11/2011 - Effective for claims with dates of service on and after October 14, 2011, contractors shall cover annual screening for adults for depression in the primary care setting that have staff-assisted depression care supports in place to assure accurate diagnosis, effective treatment, and follow-up. This revision [to the Medicare National Coverage Determinations Manual] is a national coverage determination (NCD). NCDs are binding on all carriers, fiscal intermediaries,[contractors with the Federal government that review and/or adjudicate claims, determinations, and/or decisions], quality improvement organizations, qualified independent contractors, the Medicare appeals council, and administrative law judges (ALJs) (see 42 CFR section 405.1060(a)(4) (2005)). An NCD that expands coverage is also binding on a Medicare advantage organization. In addition, an ALJ may not review an NCD. (See section 1869(f)(1)(A)(i) of the Social Security Act.) Effective date 10/14/2011. Implementation date 12/27/2011. ([TN 139](#) [↗](#)) (CR7637)

National Coverage Analyses (NCAs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with NCAs, from the National Coverage Analyses database.

- [Original Consideration for Screening for Depression in Adults \(CAG-00425N\)](#) [↗](#)

Additional Information

Other Versions

Title	Version	Effective Between	
Screening for Depression in Adults	1	10/14/2011 - N/A	You are here

